

AMEND / Amend Flow

Operational Resolution: Strategic Synthesis and Immediate Product Brief

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Source basis: Supplied strategy brief and AMEND product instructions [1] [2]

Product decision: Amend Flow should not compete to be a more attractive AI-workflow layer. Its initial job is to make unresolved operational outcomes visible, explain the failed or missing state transition, preserve the human path to resolution, and verify whether the intended real-world outcome occurred.

1. The decision that should govern this product cycle

AMEND's strongest current thesis is **Operational Resolution**, not task orchestration. In the initial adult inpatient post-acute discharge workflow, the product's value is not that it can create a follow-up, route a message, summarize a chart, or mark work complete. Those are useful enabling capabilities, but they are increasingly available within incumbent platforms and can be commoditized. The central question is instead:

What outcome should have occurred, which state should have happened next, what actually happened, where did the pathway deviate, what dependency remains, who can move it, and has the outcome truly occurred? [2]

The operating principle is therefore **COMPLETED ≠ RESOLVED**. A case can contain completed actions, approved authorization, delivered documentation, and a transport request while the patient has not departed or the hospital has not regained capacity. The case remains open until the specific operational outcome is verified—not merely until a work item is closed. [1] [2]

This is a better strategic center because it places AMEND on the layer of **outcome reconstruction and resolution learning** across systems, departments, shifts, and external partners. It should not be positioned as a claim that other vendors lack overlapping functions. It is a claim that AMEND can be designed around a more demanding object: the relationship between expected states, observed states, interventions, and verified outcomes.

Strategic question	AMEND answer	Product implication
What is the atomic object?	A patient-linked ResolutionCase , not a loose task list.	All major surfaces read and write one canonical state.
What is the core user question?	“Why is this still unresolved?”	The answer must name the missing transition, dependency, owner, authority, evidence, and next human move.
What counts as completion?	An action was performed or a message was sent.	Preserve as evidence, but do not treat it as resolution.
What counts as resolution?	The intended operational result has been observed and verified.	Maintain an explicit verified-outcome state.
What is the current operating architecture?	Amend Action Graph.	Retain it as the system connecting barrier, dependency, ownership, action, escalation, evidence, resolution, and outcome.
What may become the long-term asset?	Resolution Intelligence.	Learn, with sufficient evidence and governance, how failure patterns recover over time.

2. Source reconciliation and decisions

The supplied strategic brief consistently argues for an expected-versus-observed state model, a narrow high-friction pilot, and restraint on feature expansion. The current AMEND project instruction formalizes those choices into the product architecture. They are aligned on the substantive strategy.

There is one material naming conflict. The earlier strategy brief refers to **Cortex** and **Cortex Flow**. The current project instruction names the company **AMEND** and the product **Amend Flow**. This synthesis treats the current AMEND instruction as authoritative and regards the earlier terms as superseded. The earlier brief’s proposed **Resolution Path** should be treated as a signature interaction within a ResolutionCase—not as a new product module or a competing source of truth. [1] [2]

A second distinction requires discipline. The brief introduces a future “Resolution Graph,” while the project instruction says not to rename the current Action Graph prematurely. The

correct near-term language is:

- **Amend Action Graph:** the present operational architecture used to organize active cases and human work.
- **Resolution Intelligence:** the future differentiated capability, if validated, to learn which patterns and interventions restore outcomes.
- **Resolution Graph:** a future conceptual learning model, not a prototype or customer-facing rename in this cycle.

3. Product definition for the first demonstrable workflow

The initial workflow remains **adult inpatient post-acute discharge resolution**. It should concentrate on aged or high-risk cases where the patient's progression depends on cross-functional and external transitions such as facility acceptance, authorization status, DME, transport, receiving handoff, departure logistics, and downstream capacity restoration. [2]

This scope is intentionally narrower than a command center, prior-authorization product, referral marketplace, or transportation platform. AMEND can consume statuses from those systems as observations, but should not attempt to replace them. The design-partner proposition is accordingly modest and testable: AMEND helps a team manage a small cohort of the hardest unresolved operational exceptions while routine work stays in existing channels. [1]

3.1 The signature ResolutionCase model

Each ResolutionCase should express the following causal chain in human-readable language:

Desired Outcome → Expected State → Observed State → Exception → Dependency → Intervention → Resulting State → Verified Outcome [2]

The case must separately represent three kinds of readiness. This avoids an unsafe and misleading implication that operational coordination determines clinical appropriateness.

Readiness dimension	Product meaning	Example observation	Guardrail
Clinical readiness	A clinical team's recorded readiness state.	"Clinical team documentation indicates medically ready."	AMEND does not determine medical necessity, treatment, or discharge appropriateness.

Operational readiness	Internal operational prerequisites for progression.	Required documents complete; departure sequence initiated.	A completed item is evidence, not a verified outcome.
Disposition readiness	External destination and handoff conditions.	Receiving facility accepts; bed confirmed; handoff received.	AMEND surfaces the state and its provenance; humans retain placement decisions.

Likewise, the product must not collapse accountable follow-up into the ability to remove a constraint. **Owner** identifies the person or team responsible for moving the case forward. **Authority** identifies the person, partner, or system capable of removing the blocker. For example, a case manager may own the follow-up while the receiving facility has authority to confirm acceptance.

4. The one golden path to prototype now

AMEND should build one credible, synthetic scenario rather than a generalized workflow engine. The following path demonstrates the thesis without expanding scope.

Synthetic scenario: post-acute discharge remains unresolved after authorization

Desired outcome: The patient has departed to an accepted post-acute destination, the receiving handoff is complete, and the hospital capacity consequence is observed.

Dominant conclusion shown to the user:

Still unresolved — receiving facility acceptance has not been observed.

Causal explanation: Three prerequisite actions are complete, including a recorded authorization approval. The next expected operational state was receiving-facility acceptance by 1:30 PM. No acceptance evidence has been observed. The case therefore remains open; authorization completion is not resolution. [1]

Clear next move: The designated owner confirms current acceptance status through the established channel, records the response, and—if the predefined threshold is crossed—submits a human-reviewed escalation. The product can draft the follow-up or escalation; it must not autonomously choose a destination, decide appropriateness, or make a coverage decision.

Sequence	State or event	Classification	Case interpretation
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1	Discharge pathway initiated for a post-acute destination.	Desired outcome / context	Establishes the operational end state; does not determine clinical appropriateness.
2	Authorization approval received.	Observed state; dependency may be satisfied	Important progress, but not a case-resolution event.
3	Receiving-facility acceptance expected by 1:30 PM.	Expected state	The pathway' s next observable transition.
4	No acceptance evidence recorded at 1:30 PM.	Exception	Expected transition not observed.
5	Receiving facility has not responded or has not confirmed.	Dependency	Partner actionable; owner and authority differ.
6	Case manager confirms status; leader reviews escalation if needed.	Intervention	Action completion must be timestamped and evidenced.
7	Facility acceptance observed.	Resulting state	The case advances to transport confirmation, not to resolved.
8	Transport confirmed and departure occurs.	Progression events	Necessary, but capacity effect may remain outstanding.
9	Receiving handoff and capacity-restoration signal observed.	Verified outcome	ResolutionCase can be resolved only when the defined outcome criteria are met.

The next expected state should update only after an observed transition, or after a human records an explicit exception/replan. This preserves causal integrity: a planned action is not treated as proof that the underlying state changed.

5. Required experience: “Why is this still unresolved?”

The signature interaction is not an AI chat experience. It is a contextual explanation generated from the canonical ResolutionCase, with evidence and human review available at every material claim. The initial response should be readable in approximately 30 seconds and follow the Resolution Editorial hierarchy.

Hierarchy	What the screen must communicate	Example in the golden path
1. Dominant conclusion	Current condition and severity.	“Unresolved: facility acceptance has not been observed.”
2. Causal explanation	Which expected transition failed, despite what completed work.	“Authorization is approved; the expected acceptance state is missing.”
3. Clear next move	Named owner, channel, escalation condition, and authority.	“Case Management to confirm. Facility Admissions has authority to accept.”
4. Progressive disclosure	Evidence, timestamps, related attempts, and history.	Referral timestamp, payer response, message attempts, named source system.

The explanation should distinguish at least five states rather than showing a single generic “status” :

State	Meaning	Must not be confused with
Action completed	A person or system performed a defined task.	Dependency satisfaction or outcome achievement.
Dependency satisfied	A prerequisite condition is observed as true.	A fully resolved barrier.
Barrier resolved	The stated blocker no longer prevents the next transition.	Patient progression.
Patient progression occurred	The patient moved to the next operational stage.	Broader hospital outcome.
Operational outcome achieved	The defined end result is observed and verified.	A closure decision based only on task status.

Actionability should also be explicit. An exception can be **internally actionable**, **partner actionable**, **requires authority**, a **genuine external constraint**, or **unknown**.

“Unknown” is a valid operational state and should prompt evidence gathering rather than fabricated certainty.

6. What to build, protect, and defer

Decision	Recommendation	Rationale
Prototype an expected-versus-observed state chain for one golden path.	Build now.	It is the smallest experience that demonstrates Operational Resolution.
Add “Why is this still unresolved?” with evidence-backed explanation.	Build now.	It makes the distinction between completed work and unresolved outcome legible.
Use one canonical ResolutionCase across Morning Brief, Hospital Now, My Work, Huddle, Escalations, Shift Handoff, patient detail, and Outcomes.	Protect as non-negotiable.	Multiple sources of truth would undermine the entire resolution model.
Preserve Action Graph, evidence provenance, human ownership, escalation, and existing-channel delivery.	Protect as architecture.	These are essential to trust, continuity, and cross-shift accountability.
Require a hospital-wide integration program.	Do not require for a pilot.	A narrow exception cohort can test value with limited observed signals.
Build a dedicated prior-authorization module, referral exchange, transport platform, or marketplace.	Do not build.	They are adjacent states or inputs, not the AMEND product boundary.
Build generic agents, broad predictive patient-flow features, digital twins, or an autonomous command center.	Defer.	They dilute the test and create feature-for-feature exposure to incumbents.

Rename Action Graph to Resolution Graph.	Defer.	The learning model remains a future concept until AMEND has validated data and repeatable evidence.
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7. Design-partner pilot: the proof AMEND needs

The first pilot should not attempt to prove that AMEND can manage the entire hospital. It should test whether a small cohort—approximately 15–30 persistent, high-risk operational exceptions—can be surfaced, reconstructed, moved, and measured with less coordination burden than the current process. That cohort size is a hypothesis from the supplied brief, not a validated operating benchmark. [1]

The pilot should establish an explicit baseline before product claims are made. AMEND should not claim that it caused every reduction in length of stay or every released bed. It should measure attributable operational signals and make the uncertainty visible.

Proof category	Candidate measure	Interpretation discipline
Resolution fidelity	Share of cases with a defined desired outcome, expected next state, evidence source, owner, authority, and verified outcome.	Tests whether the model can reconstruct the pathway.
Exception visibility	Time from missed expected state to a clearly identified exception.	Indicates detection and clarity, not necessarily a causal outcome benefit.
Coordination burden	Follow-up attempts, duplicate status reconstruction, repeated huddle discussion, and manual chart/system switching.	Compare against a documented baseline where feasible.
Capacity impact	Delay hours associated with unresolved exceptions; time from resolution evidence to capacity-restoration signal.	Report as operational association unless causal design supports a stronger claim.
Resolution effectiveness	Share of interventions followed by the expected resulting state and verified outcome.	Early substrate for Resolution Intelligence; not a recommendation engine yet.

Trust and adoption	Users' ability to explain why a case is open, name the next move, and verify the evidence.	A prerequisite to scale; collect qualitative evidence by role.
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The pilot's critical research question is not merely, "Will people use another worklist?" It is: **Can users reliably identify an outcome failure that existing task and status views make difficult to reconstruct, and does the ResolutionCase reduce the work needed to move that outcome forward?**

8. Enterprise-trust and safety boundary

AMEND must remain operational decision support. It can explain a missing operational transition, summarize evidence, identify a dependency, draft a follow-up, suggest a process-defined escalation, and show patterns for human review. It must not independently recommend diagnosis, treatment, medical necessity, discharge appropriateness, placement appropriateness, or coverage decisions. [2]

All demonstrations should use clearly labeled synthetic data. Every important state should retain evidence provenance: source, timestamp, observing person or system, confidence or uncertainty where appropriate, and linkable supporting detail. A case should never be silently closed because a task was completed; if the verified-outcome evidence is missing, its status should remain unresolved or unknown according to the explicit case logic.

9. Immediate execution sequence

In the next product cycle, AMEND should produce the golden-path ResolutionCase using synthetic data and use it to test comprehension with hospital operational stakeholders. The work should move in the following order:

1. Define the exact desired outcome and verifiable terminal evidence for the chosen scenario.
2. Specify the small state model: expected state, observed state, exception, dependency, owner, authority, intervention, resulting state, and verified outcome.
3. Design the one-screen causal hierarchy around the dominant conclusion, explanation, next move, and evidence disclosure.
4. Connect Morning Brief, My Work, Huddle, Escalations, Shift Handoff, and patient detail to the **same** ResolutionCase state rather than duplicating case logic.
5. Conduct design-partner interviews using outcome-failure moments, especially statements such as "Everything looks complete," "Who are we waiting on now?"

and “Why is this still here?” [1]

6. Instrument the limited cohort so that AMEND can assess resolution fidelity, coordination burden, and observed capacity signals without overstating causality.

10. Founder-ready positioning

A disciplined external formulation is:

Amend Flow helps hospital teams resolve high-risk operational exceptions by showing when the next expected state has not occurred, why the pathway remains open, who can move it, and whether the intended real-world outcome was actually achieved.

The statement avoids claims of unique AI capabilities, universal integration, autonomous decision-making, or a market without competitors. It focuses instead on the product behavior AMEND needs to prove.

The durable company hypothesis is more ambitious but should remain an internal test rather than a present-tense promise: as ResolutionCases accumulate across contexts, AMEND may learn which failure patterns recur, which interventions alter the pathway, and which outcomes truly recover. That is **Resolution Intelligence**. It becomes defensible only if AMEND earns trustworthy evidence, preserves causal context, and demonstrates measurable value with design partners.

References

[1]: supplied strategy brief, *Cortex / Amend Strategy Brief — August 20 and August 24, 2026* (internal task attachment).

[2]: supplied product instruction, *AMEND / Amend Flow Product Thesis and Working Rules* (internal task attachment).